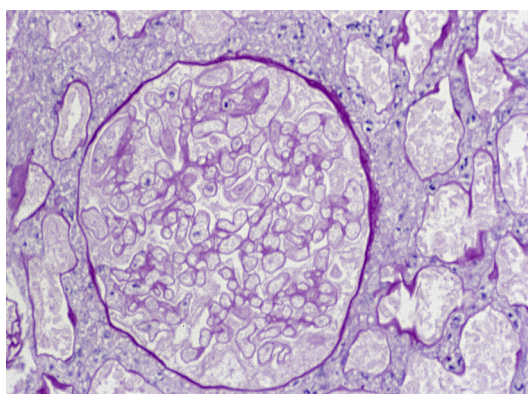


Cortical Necrosis

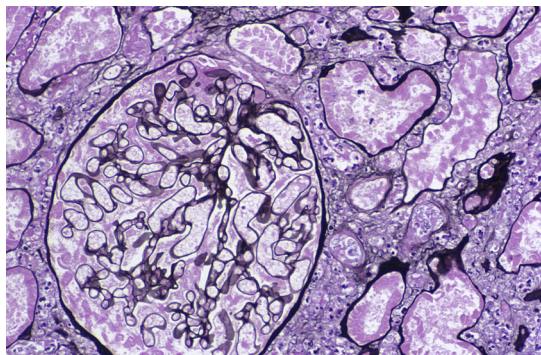
Agnes Fogo, MD

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Fig 1. Cortical necrosis is distinguished from acute tubular necrosis in that glomeruli as well as tubules show frank necrosis. It most often has a vascular occlusive etiology. Coagulative necrosis is widespread in this transplant biopsy and is manifest by small, pyknotic nuclei and ghost-like outlines of cells, characteristic of coagulative necrosis (periodic acid Schiff, $\times 400$).



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Fig 2. Cortical necrosis with tubular necrosis and glomerular necrosis. The glomerular basement membrane is still visualized, but there are no viable nuclei in the glomerulus, and surrounding tubules show frank necrosis, as do the adjacent arterioles (Jones' silver stain, $\times 400$).

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Medical Photographer: Brent Weedman.

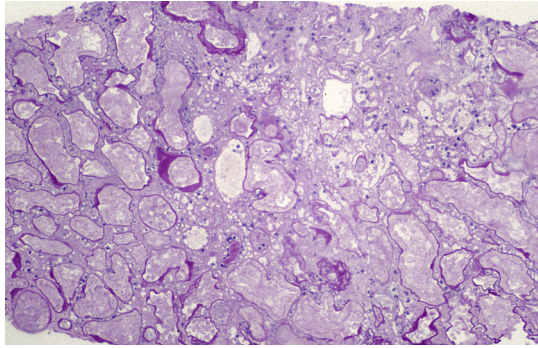
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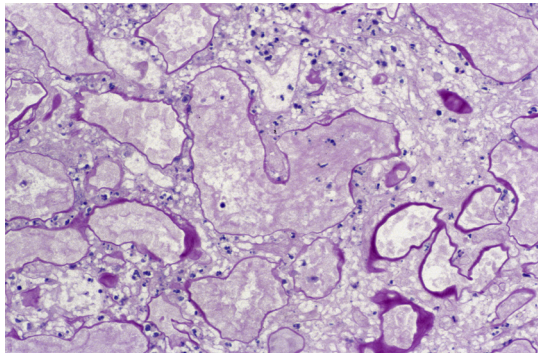
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Fig 3. Cortical necrosis with widespread frank tubular necrosis with no nuclei discernible in tubules and ghost-like outlines of cells, characteristic of coagulative necrosis (periodic acid Schiff, $\times 200$).



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Fig 4. Cortical necrosis with widespread frank tubular necrosis with no nuclei discernible in most tubules and ghost-like outlines of cells, characteristic of coagulative necrosis. There are also pyknotic nuclei and karyorrhexis, characteristic of coagulative necrosis (periodic acid Schiff, $\times 400$).